A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

20 April 2026

Dr Dejan Fiser
Medical One

Dear Dr Fiser,

Re: Miss Aimen Farhat
DOB: 1993-06-21
Address: 711/229 Franklin Street, MELBOURNE, VIC, 3000

Summary

- **Restless Leg Syndrome requiring ferritin levels above 100 ng/mL for symptom management.**
- **Referral for intravenous iron infusion at St Vincent's Private Hospital.**
- **Chronic constipation with recommendations for postural modification.**
- **Action required: Please review ferritin levels following the iron infusion and monitor Restless Leg Syndrome symptoms in conjunction with her sleep physician.**

I had the pleasure of reviewing Aimen Farhat in my rooms today. Our consultation focused on her iron stores in the context of a recent diagnosis of Restless Leg Syndrome, as well as ongoing issues with constipation.

Aimen recently underwent a sleep study under the care of her sleep physician, Dr Juan Mulder. The study confirmed a diagnosis of Restless Leg Syndrome and significant sleep fragmentation. To manage this, the sleep physician has targetted a ferritin level of at least 100 ng/mL. Aimen's recent blood tests demonstrates a stable haemoglobin, but her ferritin levels have fluctuated between 47 and 57 ng/mL despite regular oral iron supplementation. She denies any symptoms of gastrointestinal blood loss or melaena.

In terms of her gastrointestinal symptoms, Aimen continues to have some mild symptoms of constipation. She reports opening her bowels approximately twice per week. Although the stool consistency is often normal, the process is frequently painful and requires significant straining. She has not previously utilised any postural aids to assist with defecation.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan

The primary clinical objective is to increase Aimen's iron stores to the therapeutic range required for her neurological symptoms. While her current ferritin levels are not strictly deficient by standard laboratory markers, I am happy to facilitate intravenous iron to satisfy the requirements of her sleep physician. I have therefore organised a referral for an iron infusion at St Vincent's Private Hospital.

Regarding her constipation, I have advised Aimen on the benefits of using a footstool to achieve a squatting position during defecation. This simple postural modification aims to align the pelvic floor and reduce the necessity for straining. I have not recommended any further gastrointestinal investigations at this stage as her iron levels have remained stable on oral therapy and there are no "red flag" symptoms.

I have asked Aimen to review with you following the infusion to confirm her ferritin levels have surpassed the 100 ng/mL threshold. I hope this is ok with you. I am happy to see Aimen on an as needed basis.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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